



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : بسنت سعيد السيد محمد
المحافظة : القليوبية الحي / المدينة : شبرا الخيمة
رقم البطاقة : 29103031400824 رقم التليفون : 01141507210
العنوان : عرفات التهامي
العمر (تاريخ الميلاد) : 03-03-1991 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمدة 6 شهور
قسم العلاج :
الأورام

[14163]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: بسنت سعيد السيد محمد National ID: 29103031400824	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تعريف شخصية

بسمت
سعيد السيد محمد الجوهري
٢٠ عرفات الشهاشي - مدينة الزهراء
شبرا الخيمة اول - القليوبية

١٩٩١/٠٣/١٣ ٢٩١٠٣٠٣١٤٠٠٨٢٤

LR8775483

DMG
موسى

٢٠٢٦/٠٤ ٢٩١٠٣٠٣١٤٠٠٨٢٤

حاصله على ليسانس اداب

انثى مسلمة متزوجة
محمد نصر محمد نصر

البطاقة سارية حتى ٢٠٣٣/٠٤/٠٦

١

د. يوسف
رواس

3/1/2026



مملكة العربية السعودية
لصحة المرأة المصرية

07/30/2026

MDT FORM

Name: <u>Sayed Ekayed</u>	MRN: <u>6681</u>
Age: <u>35 Y</u>	ID: <u>29103031400824</u>
Menopausal Sta: <u>prem</u>	Phone: <u>0122474068</u>
NRS SCORE: <u>ps 1</u>	Marital Status:
BMI:	Family History:
CO Morbidity:	Reason Of MDY:

Clinical Data:

BX: Idc g2
ER: 8/8 PR: 8/8 HER2: 3 KI67:

Sonomammography:

Mets Workup:

History:

prem left breast cancer ct2 n1 md clips inserted Idc 2 e:8/8 pr8/8 her23+ ki
45% of 60% 12/2/2026
exam 5/2026 partial radiological response: rd of path mass and residual
microcalcif and related non mass enh
pt ended most rec 6: taxotere carboplatin last 11/5/2026 and 7 phesgo
underwent LEFT mls + AXILLARY CLEARANCE
+ve lymphovascular invasion
RCB-III
ypT1N2

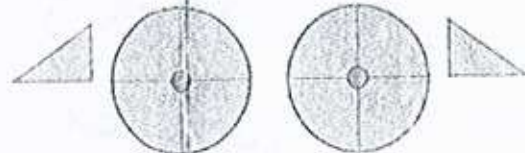
Recommendations:

MDT Outcome & Plan

In view of Non-PCR, for:
1. Kadcyla x 12-14 cycles
2. PORT to Lt breast, PCLN & Axilla
3. Adjuvant ET (Zoladex/AR) for 5 yrs
4. Zoladex q 6 mths

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric



TNM

Patient Name	بمنيت سعيد السيد محمد	Case No.	342680
Sex/Age	Female 35 Y	Patient No.	24919
Registered Date	18/7/2026 11:42:23 AM	Referred By Dr.	Out Patient
Validation Date	08/8/2026 11:17:44 AM	Branch	Gustave Roussy
Reporting Date	08/8/2026 11:22:49 AM	Pathology No.	SR-823/26

Breast Slide Revision

Clinical Data: Left breast cancer, post-NACT.

Nature of Specimen: Referred blocks & slides (Left breast wide local excision & axillary clearance).

Gross Examination:

- Twelve referred blocks labelled by (93/26). Together with, twelve referred (H&E) stained slides.

Technique:

- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
 - ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Residual viable invasive duct carcinoma with lobular differentiation.
- * Grade: 2 (differentiation 3, nuclear pleomorphism 2, mitosis 1).
- * Cellularity: 60%.
- * Ductal carcinoma in situ: Absent.
- * Lobular carcinoma in situ: Present.
- * Angioinvasion: Present.
- * Perineural invasion: Absent.
- * Microcalcification: Present.



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 Room No. (4) - First Floor-Building No. 987- Nile Corridor - Mizr El Qadema - Cairo, Egypt

Patient Name	بسمت سعيد السيد محمد	Case No.	342680
Sex/Age	Female 35 Y	Patient No.	24919
Registered Date	18/7/2026 11:42:23 AM	Referred By Dr.	Out Patient
Validation Date	08/8/2026 11:17:44 AM	Branch	Gustave Roussy
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* Resection margins: Free.

* Axillary lymph nodes: Positive viable metastatic deposits in nine out of twenty-one lymph nodes (9/21) (The largest metastatic focus is 1.2 cm, with extra nodal extension).

Results:

* **IPT (All controls showed appropriate reactivity):**

- ER: Strong nuclear reaction in 85% of tumor cells (score 8/8).
- PR: Weak nuclear reaction in 5% of tumor cells (score 3/8).
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (Ultra-low, score 0+).
- Ki67%: 2%.

Diagnosis:

Left breast cancer (post-NACT), Referred blocks & slides (Left breast wide local excision & axillary clearance):

- Residual viable invasive duct carcinoma with lobular differentiation, Grade 2 (Cellularity 60%).
- Lobular carcinoma in situ: Present.
- Ductal carcinoma in situ: Absent.
- Resection margins: Free.
- Axillary lymph nodes: Positive viable metastatic deposits in nine out of twenty-one lymph nodes (9/21) (The largest metastatic focus 1.2 cm, with extra nodal extension).

**** Stage; ypN2a.**

IPT:

- ER: Positive (8/8).



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Patient Name	بسنت سعيد السيد محمد	Case No.	342680
Sex/Age	Female 35 Y	Patient No.	24919
Registered Date	18/7/2026 11:42:23 AM	Referred By Dr.	Out Patient
Validation Date	08/8/2026 11:17:44 AM	Branch	Gustave Roussy
Reporting Date	08/8/2026 11:22:49 AM	Pathology No.	SR-823/26

- PR: Positive (3/8).
- Her2: Ultra-low (score 0+).
- Ki67%: 2%.

* *Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* **Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.



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Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناتيونال - مصر - معمل الباثولوجي



Patient Name	بسنت سعيد السيد محمد	Case No.	342680
Sex/Age	Female 35 Y	Patient No.	24919
Registered Date	18/7/2026 11:42:23 AM	Referred By Dr.	Out Patient
Validation Date	08/8/2026 11:17:44 AM	Branch	Gustave Roussy
Reporting Date	08/8/2026 11:22:49 AM	Pathology No.	SR-823/26

Signature
Prof. Dr Maha Guimei.
Dr. Walaa Ibrahim Ghonaim, MD.



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MDT FORM

3/2/2026 G

Name	د. محمد السيد	MRN	٢٩١٠٣٠١٤٠٠٨٤٤
Age	34	ID	31141507210
Menopausal status		Phone	01226353535
NRS SCORE		Marital status	متزوج
BMI		Family history	no (4 cases)
CO morbidity	medically free	REASON OF MDT	

Clinical Data

BX:	± DC	AT	R	L			
ER:	+	PR:	+	HER 2:	+	KI67:	50%
Sonomammography:							
L + VOA mass 2.2 x 2.1 cm at 12 o'clock							
Mets Workup: with architecture distortion in VOA showing suspicious micro-calcification							
TNM 210							

Recommendations: with left axillary LN (BRINOS)

Doc ✓ CESM
Unifocal Mass, 2.2 x 2.1 cm

MDT OUTCOME & PLAN

- ① Clip insert (Mass + intramammary)
- ② NACT (Docetaxel / Carboplatin / Phospho)
- ③ BCS + SLNB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name : الأستاذة / بمنت سعيد السيد
Patient No. : 29103031400824
Path No. : SR-49/26
Sex : Female
Age : 34 Y
Referring Doc :

Request No. : 13004447
Request Date : 12/01/2026
Verification Date : 14/01/2026
Financial Category : نفقة الدولة - جوسف روسي
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Left breast mass.

Nature of specimen
Block & slide revision.

Gross examination
One referred paraffin block (3848) & five referred slides were received .

Microscopic examination

- * Histological type: Mixed Invasive lobular & duct carcinoma.
- * Grade (differentiation + nuclear pleomorphism + mitosis): (2+2+2) Grade 2.
- * DCIS: Absent.
- * Lobular intra epithelial neoplasia : Positive
- * Angioinvasion: Absent .
- * Microcalcification : Positive.
- * Perineural invasion: Absent
- * IPT :
- * ER: Strong nuclear reaction in 90 % tumor cells (Allred score 8/8)
- * PR: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
- * HER2: Strong complete membrane reaction in >10 % of tumor cells (3+)
- * Ki 67: 45 %

Diagnosis

LEFT BREAST MASS, BLOCK & SLIDE REVISION

-Mixed Invasive lobular & duct carcinoma Grade 2 (tubular differentiation 2, pleomorphism 2, mitosis 2) (according to Nottingham grading system of breast cancer) .

Verified by :
Dr.Heba Hasan

Printing Date : 03/02/2026 11:19:59

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Printing By : Mrs.Mayada Hegazy



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Patient Name : الأستاذة / بسنت سعيد السيد
Patient No. : 29103031400824
Path No. : SR-49/26
Sex : Female
Age : 34 Y
Referring Doc :

Request No. : 13004447
Request Date : 12/01/2026
Verification Date : 14/01/2026
Financial Category : جوستاف روسي - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 8/8).
- * HER2: Positive (score +3).
- * Ki67: 45 %
- * Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :

Dr.Heba Hasan

Pat. Name Mrs. Basant Saced Al Sayed
Mohamed

Pat. ID: 171626

Ref. Doctor: Patient Request

Date : 24/12/2025

BILATERAL MAMMOGRAPHY:
TECHNIQUE:

- # Both breasts were imaged in different craniocaudal and mediolateral views.
- # Ultrasound was also done.

Findings:

- Normal size, shape and configuration of both breasts with normal skin, subcutaneous fat, nipples and areolar regions.
- Even fibro-glandular and fatty elements distribution in both breasts.
- Left breast showed an irregular ill-defined mass/ architecture distortion in UOQ showing suspicious micro-calcification.
- By ultrasound, an irregular hypoechoic mass lesion is seen in upper outer quadrant of left breast (12 o'clock) away from nipple by about 8cm, the lesion measured 2.2x2.1cm showing irregular spiculated margins and scattered tubular microcalcific echogenic foci.
- Also suspicious left axillary lymph nodes were noted measuring in range of 1.9x1.5cm - 2.4x1.6cm, showing lobular outline, with eccentric hypoechoic cortices
- No suspicious masses or cystic changes seen on right breast.
- Small right axillary lymph nodes are noted; largest measured 15x6 mm with preserved oval shape and central fatty hilum.
- No evidence of duct-ectasia on either side.
- No macro calcification detected.

IMPRESSION:

- # Left breast upper outer quadrant malignant-looking mass lesion and suspicious left axillary lymphadenopathy; BIRADS 5 as described.
- # Normal right sono-mammogram.

Yours sincerely,
Dr. Hani Ishak, FRCR



PATIENT NAME: Basant SAYED ELSAYYED MOHAMED

GENDER: Female

DATE : 15-Jan-2026 12:49 PM

PATIENT ID: 20226027538

AGE: 34 y

ORDER: 3727400

WHOLE BODY FDG PET-CT EXAMINATION**Clinical background:**

- 34 year-old female patient.
- Presented with pathologically proven left ILC.
- Weight = 78Kg.
- BL glucose level = 104 mg/dl.

Reason of study:

- Initial staging study.

Radiopharmaceutical:

- F-18-FDG 7.1 mCi injected.

Technique:

- Approximately one hour after IV injection of the radiopharmaceutical, whole body PET-CT examination from the skull to mid-thigh was performed with arms extended above head.
- Non-contrast CT was acquired for the purpose of attenuation correction and anatomical localization.
- No IV contrast was given.
- Data was processed and displayed according to department protocol.
- Calculation of SUVmax was done at region of interest.
- Reference hepatic activity (SUV max)~ 2.59.

Findings:

Reviewing PET, CT and fused PET-CT images:-

I- Breasts and axillary regions:

- **Left breast:** metabolically active (FDG avid) UOQ speculated soft tissue mass is seen measuring about 2.5 x 3.5 cm with SUV max~4.35. No other FDG avid focal sizable nodules/masses.
- **Few left axillary level I & II metabolically active (FDG avid) lymph nodes** the largest is seen along level I measuring 1.3 x 2.4 cm, SUV

PATIENT: Basant SAYED ELSAYYED MOHAMED, 20226027538, Female, 34 y

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مركز بهية الهم
4 شارع طلوبة أول شارع الهم بجوار داي، الجيزة

مستشفى بهية الشيخ زايد



PET Scan

CamScanner

[[DOCTYPE:PET_SCAN]]

max~7.28.

- No right breast metabolically active (FDG avid) lesions.
- No significant FDG uptake in the right axillary lymph nodes.
- Few metabolically active Subcentimetric left supraclavicular lymph nodes, the largest measures about 0.2 x 0.4 cm.

II. Head and neck:

- Diffuse symmetrical tracer uptake is seen along the examined brain with no metabolically active (FDG avid) lesions could be detected.
- No metabolically active (FDG avid) lymph nodes.
- No metabolically active (FDG avid) neck masses.
- Physiological symmetrical tracer uptake at salivary glands with no metabolically active (FDG avid) lesions.
- Intact laryngeal skeleton.
- No FDG avid thyroid nodules.

III. Chest:

- No FDG avid mediastinal or axillary lymph nodes.
- No FDG avid lung nodules or masses detected.
- No metabolically active pericardial or pleural lesions detected.
- Physiologic FDG uptake is noted in the heart.
- Aberrant right subclavian artery.
- Low grade prominent thymic tissue.

IV. Abdomen and Pelvis:

- Physiologic diffuse FDG uptake is seen over average sized liver. No FDG avid focal lesions detected. No intra-hepatic or extra-hepatic biliary radical dilatation.
- Physiologic diffuse FDG uptake is seen over average sized spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.
- Normal appearance of both kidneys with no stones or back-pressure changes. Physiologic FDG excretion is seen in the kidneys and bladder.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renal with no FDG avid lesions detected.
- Anteverted uterus with IUD seen within.
- No evidence of ascites or abdominal collections noted.

Musculoskeletal:

- Bilateral acetabular and left inferior pubic ramus few small sclerotic non metabolically active lesions, likely bone islands.
- Otherwise, normal FDG activity is seen over the axial skeleton. No focal active blastic or lytic bone lesions detected.

Conclusion:

**** [18F] FDG PET/CT study for a known case of left breast malignancy revealed:**

- **Left breast metabolically active (FDG avid) neoplastic lesion with**

PATIENT: Basant SAYED ELSAYYED MOHAMED, 20226027538, Female, 34 y

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مركز رعاية الأورام
4 شارع علوية أول شارع الهرم بجوار نادي الجزيرة

د. مصطفى هبة الشاذلي زائد
أ. عبد الأول - الشيخ زايد أمام كلية الهندسة



- *ipsilateral metastatic axillary lymph nodes.*
- *Otherwise, no metabolically active (FDG avid) metastatic lesions seen all over the rest of the body.*

Reported By,
Dr. Menna Zaki, MSc
Dr. Mohamed Mohsen, MD
Prof. Huda Fathy, Nuclear Medicine Consultant

17-Jan-2026 04:33 PM

PATIENT: Basant SAYED ELSAYYED MOHAMED, 20226027538, Female, 34 y
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مركز رعاية الأورام
4 شارع علوية أول شارع الهرم بجوار نادي الجزيرة

د. منة الشيب زكي
د. هادي الأول - الشيخ زايد لأمراض كبد الهندسة



CamScanner

PET Scan

[[DOCTYPE:PET_SCAN]]